

FORM-ADJ-15A**EOPE Blinded Case Narrative Template (PROTECT-Africa)**

ACRN Independent OAC · PROTECT-Africa (EOPE) & LOPE-Nigeria (LOPE) · Version 1.0

Purpose. Standardised, de-identified factual clinical narrative forming the body of the blinded EOPE case package (SOP-ADJ-001 §5.2; OAC Charter §6). Part A is prepared by the non-voting Adjudication Coordinator and records facts only — the Coordinator makes no classification decisions (Charter §4; SOP-ADJ-001 §4). Endpoint classification is performed independently by blinded reviewers on FORM-ADJ-02 (and FORM-ADJ-04 where discordant); Part B of this form is completed by the reviewer / OAC panel, not the Coordinator. Released only after the FORM-ADJ-01 blinding check has passed. Cross-referenced to PROTECT-Africa protocol A202501 v1.2 (eligibility §4; visits §4.9) and OAC Charter v2.0 (§9 boundary rule; definitions appendix).

BLINDING NOTICE — Do NOT record any biomarker or point-of-care result (sFlt-1/PIGF, sEng, Cerca Red, Roche Elecsys® or any POC value, category or derived risk value), site name, provider name or other direct identifier. Use the participant study ID only. PROTECT-Africa is non-interventional and these results are withheld from care and from adjudicators until database lock.

PART A — COORDINATOR-PREPARED BLINDED NARRATIVE (facts only). Sections 1–13. Records what the source documents show. No endpoint classification is made here.

1 Case identification

Study	PROTECT-Africa (EOPE)
Case ID (participant study ID)	
Site code (blinded)	
Cohort	
Prediction window(s) in scope	
Narrative prepared by (Coordinator)	
Date prepared / version	

Cohort: ☐ Derivation ☐ Validation

Prediction window(s) in scope: ☐ 1/2-week (rule-out) ☐ 2/4-week (rule-in)

2 Comprehensive pregnancy & obstetric history

Maternal age (years)	
Date of enrolment	
Gestational age at enrolment (wk+d)	
Gravidity / parity / living children	
Previous pregnancies & outcomes	
Previous miscarriage / stillbirth / neonatal death	

Relevant prior medical history (tick all that apply)

- ☐ Chronic hypertension
 ☐ Gestational hypertension / PIH
 ☐ Prior preeclampsia
 ☐ Prior eclampsia
☐ Prior HELLP
 ☐ Proteinuria
 ☐ Renal disease
 ☐ Diabetes
☐ Autoimmune (SLE / APS)
 ☐ Thrombophilia (Protein C/S, ATIII, FVL)
 ☐ None of the above

Gestational-age dating (protocol requires USS ≤ 22 weeks)

Ultrasound before 22 weeks performed? ☐ Yes ☐ No

USS date & gestational age confirmed	
LMP / dating basis	

3 Eligibility & baseline phenotype

Inclusion 1: GA 23+0 to 36+6 wk (USS ≤22 wk). Inclusion 2: singleton. Inclusion 3: clinical suspicion of PE by one or more of 3a–3e below.

Qualifying criterion under Inclusion 3 (tick all that applied)

- ☐ 3a New-onset elevated BP (SBP ≥130 / DBP ≥80) ☐ 3b Aggravation of pre-existing hypertension
- ☐ 3c New-onset proteinuria ☐ 3d Aggravation of pre-existing proteinuria
- ☐ 3e Other suspicion (symptoms / low platelets / ↑transaminases / suspected IUGR / abnormal uterine Doppler)

Eligibility pathway: ☐ Objective criterion-based ☐ Symptom-only ☐ Mixed

Exact qualifying evidence (values/findings that satisfied the criterion)

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Baseline hypertension phenotype

Classification rule — A participant on antihypertensive treatment for documented hypertension is recorded as hypertensive irrespective of the current BP. Do not record such a participant as “normotensive” on the basis of a normal Visit 1 BP (protocol §4.7; Charter definitions).

Baseline status: ☐ Truly normotensive at screening ☐ Treated gestational HTN / PIH

☐ Treated chronic hypertension ☐ Worsening chronic hypertension ☐ Superimposed-PE risk

Screening BP & repeat BP (≥6 h apart, within 1 wk)	
Antihypertensive medication history (agent, dose, start)	
Proteinuria & laboratory values at screening	
IUGR / Doppler / fetal findings used for eligibility	

4 Serial study assessments

Six scheduled visits over ~6 months. Present chronologically; record the series of results, not only the final outcome (protocol §4.9, Appendix A). Add unscheduled visits as needed.

Screening / Visit 1 (baseline)

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

Documented in record this visit: ☐ Admission ☐ Referral ☐ MgSO₄ given ☐ Delivery expedited ☐ None

Recorded symptoms · recorded maternal status · recorded fetal/neonatal status · any AE/SAE (facts from source; blinding-compliant)

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Week 1 (≤ 1 wk after V1)

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

Documented in record this visit: ☐ Admission ☐ Referral ☐ MgSO₄ given ☐ Delivery expedited ☐ None

Recorded symptoms · recorded maternal status · recorded fetal/neonatal status · any AE/SAE (facts from source; blinding-compliant)

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Week 2 (> 1–2 wk after V1)

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

Documented in record this visit: ☐ Admission ☐ Referral ☐ MgSO₄ given ☐ Delivery expedited ☐ None

Recorded symptoms · recorded maternal status · recorded fetal/neonatal status · any AE/SAE (facts from source; blinding-compliant)

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Week 4 (> 2–4 wk after V1)

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

Documented in record this visit: ☐ Admission ☐ Referral ☐ MgSO₄ given ☐ Delivery expedited ☐ None

Recorded symptoms · recorded maternal status · recorded fetal/neonatal status · any AE/SAE (facts from source; blinding-compliant)

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Delivery visit

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

Documented in record this visit: ☐ Admission ☐ Referral ☐ MgSO₄ given ☐ Delivery expedited ☐ None

Recorded symptoms · recorded maternal status · recorded fetal/neonatal status · any AE/SAE (facts from source; blinding-compliant)

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Postpartum visit (V6, ~4 wk postnatal)

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

Documented in record this visit: ☐ Admission ☐ Referral ☐ MgSO₄ given ☐ Delivery expedited ☐ None

Recorded symptoms · recorded maternal status · recorded fetal/neonatal status · any AE/SAE (facts from source; blinding-compliant)

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Unscheduled visit

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

Documented in record this visit: ☐ Admission ☐ Referral ☐ MgSO₄ given ☐ Delivery expedited ☐ None

Recorded symptoms · recorded maternal status · recorded fetal/neonatal status · any AE/SAE (facts from source; blinding-compliant)

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5 Serial summary matrix (quick reference)

Parameter	Screen/V1	Wk 1	Wk 2	Wk 4	Delivery	Postpartum
GA (wk+d)						
BP (S/D)						
Proteinuria						
Platelets						
AST / ALT						
LDH						
Creatinine						
Recorded PE status						

6 Documented maternal outcomes & diagnoses (as recorded in source)

Record what the source documents state, with dates. This is factual capture, not adjudication; the OAC endpoint classification is made by reviewers in Part B / FORM-ADJ-02.

PE-spectrum diagnosis recorded in the record

☐ None documented ☐ PE ☐ Severe PE ☐ Eclampsia ☐ HELLP

Date / gestational age at first documented diagnosis	
Days from Visit 1 to documented diagnosis (descriptive — protocol §4.6 grouping; not the OAC onset classification)	

Maternal adverse outcomes recorded (tick all; state date)

☐ Maternal death ☐ Pulmonary oedema ☐ Acute renal failure
☐ Cerebral haemorrhage ☐ Cerebral thrombosis ☐ DIC ☐ None

Detail & timing of any documented maternal adverse outcome

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7 Documented fetal / neonatal outcomes (as recorded in source)

☐ Perinatal / fetal death ☐ Delivery <34 wk ☐ IUGR ☐ Placental abruption
☐ Respiratory distress ☐ NEC ☐ IVH ☐ None

Gestational age at delivery	
Fetal growth / Doppler findings	
Birth weight (g) / centile	
Neonatal admission course (incl. NICU, neonatal death if applicable — descriptive)	
Outcome & timing	

8 Maternal clinical course (integrated factual narrative)

Chronological narrative of documented facts: symptom progression, BP progression, lab progression, timing of any recorded diagnosis, admission/escalation/treatment, maternal outcome (blinding-compliant)

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9 Concomitant medications & exposures

Medication / exposure	Indication	Start date	Stop date	Temporal relation to outcome

Categories present: ☐ Antihypertensives ☐ Aspirin ☐ Iron / folate ☐ Antibiotics ☐ Supplements / OTC

10 Protocol deviations / data-quality flags

- ☐ Missing / contradictory source data ☐ Source-to-eCRF inconsistency
- ☐ Missing delivery labs ☐ Incomplete postpartum follow-up ☐ Eligibility concern
- ☐ Timing discrepancy ☐ Missing USS <22 wk ☐ Antihypertensive / HTN misclassification

Detail of each flag (blinding-compliant)

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11 Source reconciliation table

Data item	Source document value	eCRF value	Concordant?	Resolution / query ID

12 Missing-data tracker

Field / visit	Status	Query ID	Date raised	Date resolved	Impact on adjudicability

Mark unavailable fields as [To be confirmed], [Pending] or [Not available]. Incomplete follow-up is not “no endpoint.”

13 Coordinator’s questions / uncertainties for the panel

Neutral factual questions or ambiguities to flag for the reviewers (e.g., conflicting dates, missing labs). Do not propose or imply a classification.

#	Question / uncertainty (factual)	Related field / query ID
1		
2		
3		

Role	Name	Signature	Date
Narrative prepared by (Coordinator)			
QA blinding counter-check			

PART B — REVIEWER / OAC PANEL DETERMINATION. Completed by the blinded reviewer(s) / OAC panel — NOT the Coordinator. The controlled classification of record is FORM-ADJ-02 (and FORM-ADJ-04 if discordant); this summary supports the panel brief and must be consistent with those forms.

14 Adjudicated classification (reviewer / panel use)

Primary maternal endpoint: ☐ No PE ☐ PE ☐ Severe PE ☐ Eclampsia ☐ HELLP

Onset by gestational age (Charter §9, 34+0 boundary): ☐ Early-onset (<34+0 wk) ☐ Late-onset (≥34+0 wk)

Relationship of outcomes to preeclampsia: ☐ Attributable ☐ Not attributable ☐ Uncertain

Adjudicable: ☐ Yes ☐ Not adjudicable (record reason on FORM-ADJ-06)

Rationale / criteria applied (consistent with FORM-ADJ-02)

15 Panel-ready determination summary (1-page brief)

What happened (concise clinical synopsis)

Is the endpoint met? Which endpoint(s), and which remain uncertain?

Was the participant correctly classified at enrolment (esp. HTN vs normotensive)?

Does this case suggest a broader site or protocol issue?

Role	Name	Signature	Date
Reviewer			
OAC Chair (if panel-resolved)			